

The Site and Partner Package

Two La Jolla institutions approached in parallel on the same day, two foundations asked a mechanism question, and one determination that gates every other item. No agreement of any kind exists with any institution named here.

Day 3 of 5

Repository v4.8.0

3 figures

5 tables

No agreements

The trial
18 participants
Phase 1, 3+3 escalation

The meeting
45 minutes
Five questions, one agenda

Site start-up
\$420,000
Held, nothing committed

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Independent research paper and practical adoption guide. It is not medical or regulatory advice and is not endorsed by the FDA, NIH, HHS, an IRB, ICH, or any sponsor. All figures derive from the author's repository sources and are illustrative unless tied to a cited reference.

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Repository (v4.8.0) is at <https://github.com/kevinkawchak/physical-ai-oncology-trials>. The paper files are deposited in funding/auto-fund. No digital object identifier is asserted for this document.

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Abstract

A funder buys a trial, and a trial needs a site. This packet accompanies five letters: two to La Jolla institutions approached in parallel on the same day, two to disease foundations asked a mechanism question rather than for money, and one to a clinical trial support service asking for the determination that gates every other item, which is which party is capable of serving as sponsor-investigator [1].

It sets out the five questions a 45-minute feasibility meeting has to settle, the obligations that would fall to a site, to the sponsor, and to the agent's developer, and the site start-up money that is held and not committed.

No agreement of any kind exists with UC San Diego, with Moores Cancer Center, with Scripps Research, with Scripps Health, or with any other institution. Both institutions are named at the feasibility stage only. Neither is described as a partner, sponsor, site, or endorser, and neither is described to the other as preferred [2, 3].

1 What This Packet Is For

This is the third of five daily packets in `funding/auto-fund`. Day 1 re-contacted five federal mechanisms after the approval of Rasonque on August 26, 2026 [4]. Day 2 addressed the private capital the federal route does not reach. This day addresses the thing both of those exist to fund.

1.1 The order these three days are in, and why

A site asks who is funding the work and then asks who the investigator will be. The first question is answered by day 1 and the second by the conflict boundary set out in day 2, so approaching a site before either answer existed would have produced a conversation that stalled on its second question [5].

The same logic governs what is not sent on this day. No engineering detail travels before a confidentiality agreement is in place, and the route for that agreement is asked as a question in the fifth letter rather than assumed in the first.

1.2 How to read this

A site principal investigator or a trials office should read §2, which is the meeting agenda, and §3, which is the obligation split. Those two are the whole of what a first meeting needs.

A foundation reviewer should read §4, which sets out the funnel and the three mechanisms, and §0 above for what is and is not claimed.

A funder or a holder should read §5, which shows that site start-up money is allocated, segregated, and committed to nobody. §1 explains why two institutions are approached at once, §6 carries the positioning constraints, the method note, and the references.

2 Two Routes, Run in Parallel and Disclosed

Two La Jolla institutions are approached on the same day by two different routes, asking two different questions. Each letter discloses the other. Neither is asked for exclusivity and neither is described to the other as preferred.

2.1 Why parallel rather than sequential

A sequential approach costs a quarter. A first institution takes four to eight weeks to route an unsolicited feasibility request to the right surgeon, and if the answer is no, the second institution starts from zero after that. For an eighteen-participant Phase 1 whose protocol is already complete and deposited, spending a quarter to learn a routing answer is the expensive option [6].

A parallel approach costs something too, and it is worth naming. Two institutions in one small clinical community will learn of each other's involvement. The only question is whether they learn it from the letter or from a colleague, and learning it from the letter is strictly better for everyone.

Criterion	Route one	Route two
Entry point	Escalation on a published path, after an August 2026 request	An inbound visibility notice to a digital trials group
What leads the letter	The clinical program	The artificial intelligence method
The ask	A routing decision: who is the right site principal investigator	A read of the public portfolio and a view on the method
Pancreatic surgical volume	The criterion a first meeting establishes	The criterion a first meeting establishes
Robotic platform availability	Asked, not assumed. No configuration is asserted	Asked, not assumed
Trial operations and budgeting	A trials office and a budget questionnaire route exist	A centralized research operations office exists
What neither is asked	To endorse, to host, or to commit	To endorse, to host, or to commit

Table 12. The two candidate routes against seven criteria, with the two rows that are asked rather than assumed marked by their own wording.

The asks differ because the entry points differ. Approaching a digital trials group with a request to host an experimental surgical operation would be asking the wrong department for the wrong thing, and approaching a cancer center's leadership with a method-evaluation invitation would waste an escalation that already has a published path.

2.2 Where each function would physically sit

Figure 8 answers the question a site operations lead asks first, which is not who is responsible but where the work happens. A function that is legally the sponsor's but physically on a hospital floor needs space, badge access, and a network drop, and none of that appears in an obligation diagram.

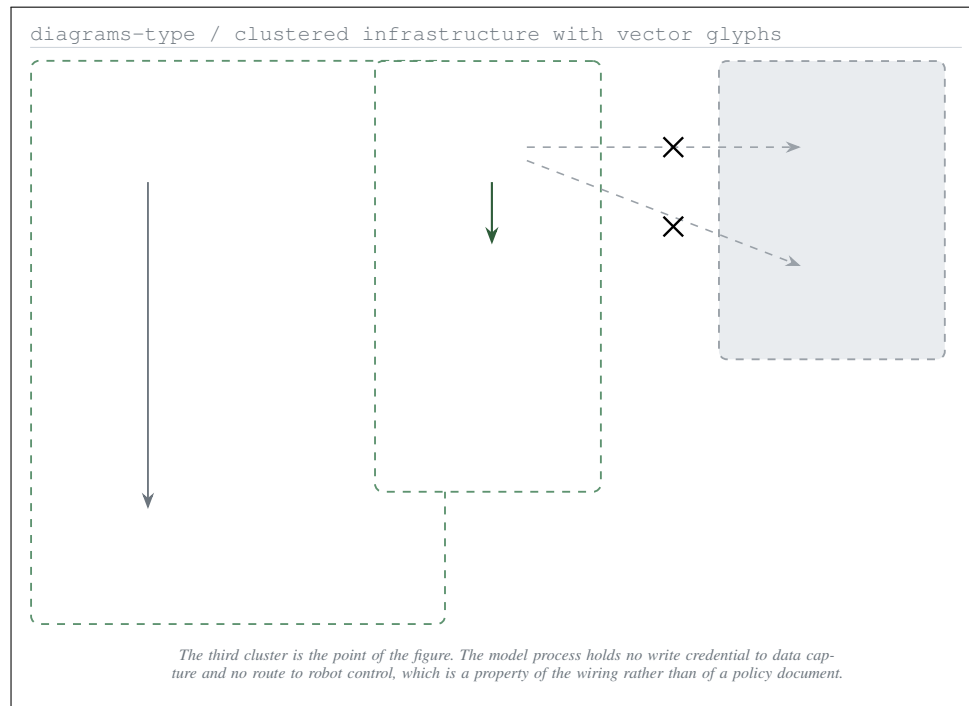


Figure 8. Where each trial function would physically sit, and the two systems the advisory model process is architecturally prevented from ever reaching.

2.3 The clinical introductions that are deliberately deferred

Two clinicians at one of the two institutions are relevant to this program by specialty, and no public professional address was located for either. This company will not guess one. If that institution responds, the introduction is requested through its own research services rather than attempted directly. The rule holds generally: an introduction requested through an institution is an introduction the institution knows about [3].

2.4 The escalation rule, applied to both

If there is no response within three business days, the original request is forwarded once, with a short note, to the contact one level up on the published path. Once, and not repeatedly. A second escalation inside a week converts a routing question into an impression of the company, and the impression outlasts the question.

3 The Five Questions a First Meeting Settles

Every question below is answerable in a 45-minute meeting by people who are already in the room. None requires a document read in advance, a committee convened, or a decision escalated. A feasibility meeting whose agenda requires preparation is a feasibility meeting that gets rescheduled.

#	The question	What a yes unlocks, and what a no costs
1	Is the population and the perioperative concept clinically supportable here	Everything else on the list. A no means stop rather than negotiate, and stopping early is the cheap outcome
2	Could a pancreatic surgeon here serve as site principal investigator, and a GI oncologist lead systemic therapy	The regulatory structure. The sponsor cannot supply this role, because every 21 CFR 54.2 trigger would fire if one person held both
3	What is the annual eligible-patient count, and what competes for it	An enrollment period a funder can believe. Three competing protocols on the same population turn an 18-participant escalation into a five-year trial
4	Is the robotic platform available, and is the workflow permissible on it	A configuration that can be named in a regulatory submission. Without one, the submission cannot be written at all
5	Would the institution host an investigator-initiated trial sponsored by a small company	The contracting, budgeting and review workstreams. It is asked last because it is most likely to be answered by policy rather than by the room

Table 11. The five feasibility questions in the order a meeting should take them, with what a yes to each unlocks and what a no to each costs.

Question 4 is where the robotic configuration gets specified. No manufacturer, model, arm count, or software version is asserted anywhere by this company: the configuration is settled at a site agreement, and this question is the conversation that settles it. What is needed from the site is which systems are available, which instruments, which software versions, what networking restrictions apply, what cybersecurity requirements apply, and whether a recorded research session is permissible.

3.1 The one item that gates all five

Which party is capable of serving as sponsor-investigator. The applicant, the site principal investigator, the FDA sponsor, the investigational new drug application holder, and the device manufacturer do not have to be the same entity, but their responsibilities have to be explicit, and today they are consistent only with this company's assumption about them [7, 1].

That determination is requested separately from clinical trial support services, because it is a regulatory question rather than a clinical one and it should not consume a clinical meeting.

3.2 What is brought to the meeting, and what is not asked for

A Phase 1 protocol, an investigational new drug application, a Phase 2 protocol, quantitative simulations with their verification notebooks, and a fifteen-document site package are complete, public, and deposited [6, 8, 9, 10, 11]. The budget frame is \$700,000 per year and \$3,500,000 over five years in direct cost [12], and \$420,000 of private capital is allocated to site start-up and committed to nobody.

What is not asked for in a first meeting: an endorsement, a hosting commitment, a letter of support for a grant application, a decision from senior leadership before a surgeon and an oncologist have formed a view, or the sharing of any confidential engineering detail before a confidentiality agreement is in place.

3.3 What happens in the week after the meeting, by outcome

A feasibility meeting has three possible outcomes and each has a different next move, written now so that none of them costs a week of deliberation later.

If the answer to questions 1, 2 and 5 is yes, the concept intake pack in `funding/auto-fund/04Sep26/forms` is submitted with the named investigator, the budget questionnaire is returned to the trials office, and stage 1 of the site start-up draw in §5 releases against the written response.

If the answer to question 1 or 2 is no, the right move is to stop with that institution rather than to negotiate. A population that a pancreatic surgical oncologist judges unsupportable is not made supportable by a second conversation, and the second route of §1 exists so that a no is a redirection rather than a dead end.

If the answers are yes but question 4 cannot be answered because a platform is unavailable, the study is not blocked: the configuration is a site-agreement item and a different site may hold a different platform. What that outcome does block is the regulatory submission, which cannot name a configuration that has not been identified, so it is recorded as an open item rather than as a failure.

3.4 The single approval step

One decision is open on this day

Approve naming a target week for a feasibility meeting, and approve asking two institutions and two foundations for related things on the same day. Naming a week is the whole of it: a request with no proposed date becomes a thread, and a request with a proposed week becomes a calendar entry or a clear no. Both are more useful than a thread. Every letter in `funding/auto-fund/04Sep26/emails` carries a bracket for that week and must not be sent with the bracket unresolved.

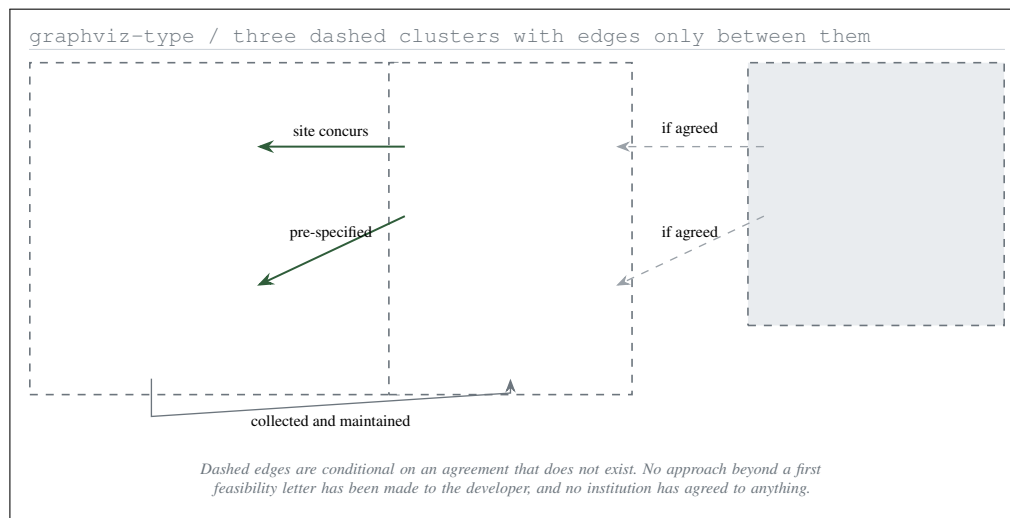


Figure 7. Site, sponsor and developer obligations as three disjoint sets, with edges only between them and dashed lines wherever no agreement yet exists.

4 Who Is Responsible for What

A site agreement is blocked more often by an unanswered responsibility question than by a scientific one. This section states the split as it would stand, marks everything conditional as conditional, and lists the four questions that are open.

4.1 Why the site holds the investigator role

The chief executive is chief executive of the sponsor and holds the whole of it. He is not a clinical investigator on this trial, will not be one, and is nowhere described as one. If he held both roles, all four of the 21 CFR 54.2 disclosable interests would fire at once: compensation influenced by the study outcome, significant equity in a non-publicly traded sponsor, a proprietary interest in the workflow assets, and significant payments of other sorts [5, 13].

Because all four would fire, the separation cannot be partial. A Phase 1 3+3 escalation carries no randomization, so the boundary binds on dose assignment, dose-limiting toxicity adjudication, and the analysis plan instead, and the wording is carried forward so that it still binds when a Phase 2 successor introduces randomization [9].

4.2 The three corrections every letter carries

#	The correction	Why it travels with every communication
1	Daraxonrasib is not described as first in human	It is approved in the metastatic setting and was in Phase 3 evaluation before that. The supportable claim concerns the integrated surgical and advisory workflow
2	The robotic configuration is specified at the site agreement and not before	No site can assess feasibility or regulatory status without an identifiable platform, and asserting one this company does not have would be worse than asserting none
3	No institution is a partner, sponsor, site, or endorser	No agreement of any kind exists, and a single sentence implying otherwise would be found by the institution named in it

Table 15. The three positioning corrections carried into every letter, with the reason each one travels rather than being stated once and filed.

4.3 The four questions this section leaves open

Who holds the investigational new drug application, the sponsor or the developer. Whether a site physician could serve as sponsor-investigator, and whether that would be better or worse than the current split. Who performs safety reporting, monitoring, pharmacovigilance, drug accountability and annual reports. And who pays for insurance, indemnification, subject injury, and device and software validation [14].

All four are asked in the fifth letter of this day. None is answered here, because answering them in a document the institution has not seen would be answering them alone.

4.4 The confidentiality route, asked rather than assumed

No confidential engineering detail travels before a confidentiality agreement is in place, and the route that agreement should follow is a question in the fifth letter of this day rather than an assumption in the first. The project is investigator-authored, seeks federal support, has a for-profit applicant, and may require drug support from a third party, and each of those facts can change which institutional office should hold the agreement.

Asking is cheaper than guessing. An agreement executed through the wrong office is renegotiated through the right one, and the renegotiation happens at the point where the detail is actually needed, which is the worst moment to discover it.

4.5 What a reader should take from this section

That the responsibility split is written down before it is negotiated, that everything conditional is marked conditional in the figure as well as in the text, and that the four open questions are named rather than papered over. A site that receives a document asserting a split it never agreed to reads a company that will be difficult later. A site that receives one marking its own gaps reads a company that has thought about the agreement it is asking for.

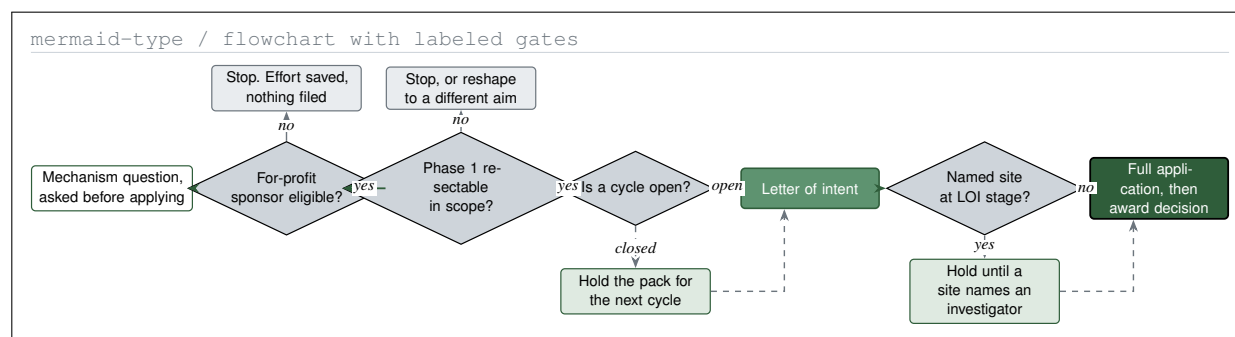


Figure 9. The foundation funnel drawn as four gates, with the exit at each and the two holds, so that the cheapest gate to fail is visible as the first one.

5 The Foundation Route

Two foundation letters go out on this day and neither asks for money. Both ask whether a mechanism can fund a Phase 1 whose sponsor is a for-profit small business with an academic clinical site under a separate agreement. That question is answerable by email in a week; the same answer discovered after a full application costs a cycle, and a cycle for a disease foundation is usually a year.

5.1 Three mechanisms, and what each would have to answer

Mechanism class	Cycle	The eligibility question that decides it
Disease foundation research grant, pancreatic cancer specific	Published annual cycle	Whether the applicant of record may be a company with a named academic site, or must be the institution
Pre-competitive consortium, public and private partners	Rolling, by consortium formation	Whether a single-company applicant can join a mechanism designed for multi-party consortia
Institutional pilot award at a candidate site	Annual, applied for by the site	Whether a site investigator would apply, which is a site decision and not this company's

Table 13. Three foundation mechanism classes with their cycle shape and the single eligibility question that determines whether an application fits.

The third row is the one this company cannot act on alone. An institutional pilot award is applied for by the institution, so it becomes available only after the feasibility conversation of §2 has produced an investigator who wants it. It is listed here so that it is not forgotten at the moment it becomes possible [2].

5.2 What a foundation is told about the evidence

The same six quantities that go to a federal reviewer, each carrying the limitation its own authors stated: a ten-arm quantitative systems pharmacology run at 12.8 months median overall survival against 5.4 [10], the RASolute 302 readout at 13.2 against 6.6 [15], a 1000-patient digital twin at 12.1 months and a progression-free hazard ratio of 0.31 [6], a credibility score of 81.9 across 55 verification tests against the ASME V and V 40 framework [16], and an empirical triplicate across 100,000 records [17].

Three of the six are in silico or digital twin results and are labeled as such in every table that carries them. The proximity between the 2025 simulation and the 2026 trial readout is a chronology observation and a hypothesis-supporting one, not a validation claim, and the sample size, single-agent and molecular-selection differences are material.

6 Site Start-Up Money, Allocated and Committed to Nobody

\$420,000 of private capital is allocated to site start-up. **Nothing has been committed.** No agreement of any kind exists with any institution, and a company that commits start-up money to an institution that has not agreed to anything has converted capital into enthusiasm.

Stage	Ceiling	What it pays for	The artifact that releases it
1	\$60,000	Confidentiality agreement, contracting route determination, the first coordinated start-up call	A written response from a clinical trial support service
2	\$110,000	Budget questionnaire response, coverage analysis, the institution's start-up fee	A named site principal investigator and a returned budget
3	\$150,000	Institutional review board submission and regulatory package assembly	An executed site agreement
4	\$100,000	First coordinator and regulatory time before enrollment opens	Institutional review board approval
	\$420,000	From the private capital use of proceeds	Four artifacts, none of which this company produces

Table 14. Four draw stages with a ceiling on each, and the artifact that releases each one, every artifact produced by somebody other than the sponsor.

Each trigger is an artifact somebody else produces. That is deliberate. A stage that releases on a date releases whether or not the work it funds is possible; a stage that releases on an artifact cannot.

6.1 Why this pool is held short and not laddered

The operating reserve is laddered because its draws are known in advance [18]. This pool's draws are not: their timing depends on when a site names a principal investigator, when a budget questionnaire comes back, and when an institutional review board meets. A ladder against unknown draws guarantees that a maturity eventually has to be broken to pay an invoice, which is the loss a ladder exists to prevent. So the pool sits at seventy percent in a government sweep, twenty percent in one short-duration Treasury fund entered on a limit order, and ten percent in a single three-month bill. The yield given up is the price of not being forced.

6.2 How this reconciles to the program budget

The five-year program is \$700,000 per year and \$3,500,000 in direct cost, and site personnel sit inside the \$521,000 personnel line of that frame [12, 11]. This pool is one time, is private capital rather than federal award funds, and pays for start-up work that precedes an award.

That last point is the one a program officer would check. Start-up work performed before an award start date is not chargeable to that award, which is precisely why it is funded privately here, and why federal award funds and private capital are never commingled in either direction.

6.3 What a site is told about this in a first meeting

That the money exists, that it is segregated, that it is committed to nobody, and that the first stage releases on a written response rather than on a signature. A site that hears a number in a first meeting hears seriousness; a site that hears a number and then discovers the number was aspirational hears something else, which is why the ceilings in Table 14 are ceilings rather than forecasts.

7 Positioning, Method, and References

7.1 Positioning constraints

No agreement of any kind exists with UC San Diego, with Moores Cancer Center, with Scripps Research, with Scripps Health, or with any other institution [2, 3, 1]. Both institutions are named at the feasibility stage only. Neither is described as a partner, sponsor, site, or endorser, neither has reviewed or endorsed anything, and neither is described to the other as preferred.

No institutional review board has reviewed anything in this program. No patient has been treated. No robotic configuration has been specified or cleared; the configuration is settled at a site agreement and not before, and no manufacturer, model, arm count, or software version is asserted anywhere.

No drug supply agreement, letter of authorization, or regulatory cross-reference is in place with the agent's developer, and the letter of the preceding day is the first approach of any kind [19].

Daraxonrasib is approved in the metastatic setting and was in Phase 3 evaluation before that [4, 15], and is nowhere described as first in human. The perioperative use this program proposes is not the approved setting, remains investigational, and still requires an investigational new drug application [7].

Nothing in §5 is investment advice or a recommendation to any other person, no order described there has been placed, and no dollar of the site start-up line has been committed. The company's comparable virtual trial work is described as **projected** at \$36,330 per run against an industry benchmark above \$120,000 per run; that figure describes simulation work already completed and deposited and is not a projection of clinical trial cost [20].

Favorable responses to project applications have been received from federal funders, three presidential recognition letters have been issued to the chief executive, and industry recipients of this company's communications have been responsive to its initiatives. Each is a fact about correspondence, and none is an award, an agreement, a commitment, or a review [11].

7.2 Method and reproducibility

This packet was produced from repository sources and nothing else: the feasibility questions and the escalation path from the partner research, the obligation split from the capitalization plan's conflict section [20], the staffing and location detail from the site documentation package [11], and the budget frame from the ten-application work [12]. The three figures are drawn in TikZ from the specifications in `funding/auto-fund/04Sep26/diagrams`; no raster image is used anywhere, and the parent style's stick-figure macro is deleted rather than left unused. Three frontier models divide the work in this company's practice, one for repository-scale production and two for independent review [21]. The human author retains responsibility for every conclusion here, and this document was adapted using Claude Code Opus 5 [22].

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